



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هيت محمد حسنين احمد
المحافظة : اسيوط
الحى / المدينة : ساحل سليم
العنوان : =
رقم البطاقة : 28702102504046 رقم التليفون : 01019950663
العمر (تاريخ الميلاد) : 10-02-1987 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و
بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: هيثم محمد حسانين احمد National ID: 28702102504046	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية

هيبث
 محمد حسنين احمد
 قرية الطاهرة
 مركز ساحل سليم - اسيوط

١٩٨٧/٠٢/١٤
 ٢٨٧.٢١.٢٥.٤٠.٤٦
 LQ1773274


 ٢٠٢٦/٠٢
 ٢٨٧.٢١.٢٥.٤٠.٤٦
 ربة منزل

أنفى
 مسلمة
 متزوجة
 أحمد سيد عطيه عبدالعال
 البطاقة سارية حتى ٢٠٢٢/٠٢/١٤







Presidential Initiative for Women's Health

Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: هيثم محمد حسنين احمد

Patient's I.D : 28702102504046

Age: 32 Y.

Pathology Number: 67/2026

Specimen Received Date: 28 / 03 /2026

Date Report Issued: 31/ 03 /2026

Clinical data: Left breast mass.

Nature of the specimen: U.S. Guided Left Axillary L.N, Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed fibrofatty tissue fragments admixed with cores of lymphoid tissue with preserved portions of capsule and patent subcapsular sinuses. They showed polymorphus lymphoid population with patent sinuses plugged by histiocytes with scattered melanin laden forms and many neutrophils. Focal fibrosis was seen. No evidence of granuloma or malignancy detected within the limits of received biopsy

DIAGNOSIS: U.S. Guided Left Axillary L.N, Tru Cut Biopsy:

- Lymphoid Tissue Cores Negative For Metastatic Lesion; Features Compatible With Chronic Non-specific Lymphadenitis To Be Confirmed By Immunophenotyping Using (CD3, CD15, CD20, CD30, Ki67,etc) if needed.

PROF. DR. SOMIA ABDEL LATIF
Dr. MIRAN ATIF

Sonia
MIRAN

Name: MR. :	هيت محمد حسانين احمد 10465	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	
	رقم طبي:	
إيماء للطلب المقدم من بقسم:		
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	
تاريخ وساعة الدخول :		
وبعد الكشف والفحوصات والمتابعة وجد انه 38 years old patient was diagnosed with DCIS on 11/25. A grade 3 lumpectomy was performed the same day. The resection margins were involved. ER 5/8, PR 4/8, Ki-67 60 %. MRI of the breast revealed a 7.7 × 6 cm lesion in the left breast with the shortest distance to the skin of 1 cm and a smaller 2.7 cm lesion. A 3.2 cm left axillary lymph node was noted. Biopsy of the breast mass showed a tiny focus of stromal invasion; the axillary lymph node biopsy was pending. A CT of the chest and abdomen with contrast and a bone scan were ordered. Axillary lymph node pathology and CT results were pending; the bone scan had not been performed because it was not approved. The patient was re-consulted and noted to have an inflammatory breast lesion with an ulcer and purulent discharge around the nipple, for which antibiotics were prescribed. The patient has a left breast cancer staged as cT4b N2. A left axillary lymph node biopsy at Gustave was IDC 3, ER 0/8, PR 0/8, HER2 + . At Kasr on 11/2025, a left breast mass was biopsied twice: one DCIS (ER 5/8, PR 4/8) and one triple-negative. MDT discussion recommended 4 cycles of AC (doxorubicin + cyclophosphamide) and 4 cycles of THP (paclitaxel + carboplatin + trastuzumab) with an estimated 62 % response by 6/2025. On examination, the left breast showed edema, redness, induration, and an ulcerative lesion at the nipple. A left axillary lymph node measuring 3 × 3 cm was noted for c1 AC and ADRA 106, with endoxan 1060. On examination, the left breast continued to show edema, redness, and ulcerative lesions at the nipple. The left axillary lymph node measured 2 × 3 cm and was noted for c3 AC and ADRA 106, with endoxan 1060 as neoadjuvant therapy.		
تحريراً في ,		
	الوقت :	
التاريخ : 21-09-2026		
الطبيب المعالج : dina makram		
يعتمد ,		



Presidential Initiative for Women's Health

Pathology Lab - Cairo University Hospitals

العيادة النسائية لصحة المرأة - معمل الباثولوجي

مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: أمينة محمد عبدالمعطي

Patient's I.D.: 28702102504046

Age: 39 Y

Pathology Number: 25/2026

Specimen Received Date: 02 / 02 / 2026

Date Report Issued: 17/ 02 / 2026

Clinical data: Left breast UOQ mass, BIRADs 5.

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of breast tissue, showing dilated ducts distended by moderately to markedly dysplastic cells. Tiny focus of stromal invasion was seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone, Her-2/neu (Anti-Ki67 (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (SP2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (50-5) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed negative nuclear staining in tumor cells (invasive and in situ) by chromogen.

Examination of section treated for progesterone receptors revealed negative nuclear staining in tumor cells (invasive and in situ) by chromogen.

Examination of section treated for Her-2/neu revealed negative membrane staining in invasive tumor cells by chromogen (score 0).

Examination of section treated for Ki67 revealed positive nuclear stain in average of $\pm 50-60\%$ of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Ductal Carcinoma In Situ, High Grade, Associated With Focal Stromal Invasion.
- Negative For Estrogen Receptors.
- Negative For Progesterone Receptors.
- Negative For Her-2/neu Over-Expression (Score 0).
- 50-60% Ki67 Proliferative Index.

Comment: Given the paucity of invasive tumor cells in the presented specimen, repetition of immunohistochemical stains on excision biopsy specimen is advised.

PROF. DR. AHMED AYOUB AZIZ

أ. د. أحمد عيسى



Presidential Initiative for Women's Health

Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

مستشفيات جامعة القاهرة

PATHOLOGY REPORT



Name: هيثم محمد حسنين احمد

Patient's I.D : 28702102504046

Age: 39 Y

Pathology Number: 67/2026

Specimen Received Data: 26 / 03 /2026

Date Report Issued: 31/ 03 /2026

Clinical data: Left breast mass.

Nature of the specimen: U.S. Guided Left Axillary L.N. Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed fibrofatty tissue fragments admixed with cores of lymphoid tissue with preserved portions of capsule and patent subcapsular sinuses. They showed polymorphous lymphoid population with patent sinuses plugged by histiocytes with scattered melanin laden forms and many neutrophils. Focal fibrosis was seen. No evidence of granuloma or malignancy detected within the limits of received biopsy

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PROF. DR. SOMIA ABDEL LATIF
Dr. MIRAN ATIF

Sonia
MIRAN

HISTOCARE LAB

د. أمل أحمد هريدي

Dr. Amal Ahmed Hareedy

دكتوراه تحاليل الأنسجة و الأورام و الخلايا
جامعة القاهرة القصر العيني

PATIENT'S NAME: هبة محمد حسنين

AGE: 38Y

SEX: F

REFERRED BY PROF. DR.: Gebril Omar

CLINICAL DIAGNOSIS: Left breast mass.

NATURE OF THE SPECIMEN: Excision.

SPECIMEN OBTAINED AT: 16/11/2025

SPECIMEN REPORTED AT: 23/11/2025

PATHOLOGY REPORT

GROSS: Lumpectomy measured 10x7x3cm. Serial cuts revealed an ill defined grayish white mass with dilated ducts of the whole specimen.

MICROSCOPY: Sections examined from the mass revealed markedly dilated ectatic ducts lined by proliferated moderately to markedly anaplastic ductal epithelial cells arranged in comedo & solid patterns with lobular cancerization reaching resection margins supported by fair fibrotic stroma with mild lymphocytic infiltrate. No evidence of invasion in sections examined.

DIAGNOSIS: LEFT BREAST MASS; LUMPECTOMY:

- DUCT CARCINOMA IN SITU NUCLEAR GRADE 3, MAINLY COMEDO & SOLID PATTERNS, REACHING RESECTION MARGINS
- NO INVASION.
- TNM STAGE; pTis

N.B: ER & PR Immunostains are available on request

NR: Two slides are enclosed
Thick for reference

SIGNATURE

DR. AMAL HAREEDY, MD

د. امل احمد هريدي

Dr. Amal Ahmed Hareedy

دكتوراه تحاليل الانسجة و الاورام و الخلايا
جامعة القاهرة القصر العيني

PATIENT'S NAME: هبة محمد حسنين

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- DUCT CARCINOMA IN SITU NUCLEAR GRADE 3, MAINLY COMEDO & SOLID PATTERNS, REACHING RESECTION MARGINS
- NO INVASION.
- TNM STAGE; pTis

N.B: ER & PR immunostains are available on request

NB: Two slides are enclosed
Thanks for reference

SIGNATURE

DR. AMAL HAREEDY, MD



وحدة دلالات الأورام
قسم الباثولوجيا - قصر العيني
٠١٠٦٨٥٢٩٦٨٤ : ت

IMMUNOHISTOPATHOLOGY

Patient Name: هبة محمد حسنين

lab.No:2025-1584

Date:3/12/ 2025

Age38: Y

Sex: Female

Markers: P63,ER, PR, and Ki67 .

Procedure:

Sections were prepared from paraffin blocks treated for P63,(ER-clone SP1), (PR clone 1E2), and (Ki67- clone 30-9) automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for ER revealed intense nuclear staining in <10% of tumor cells by chromogen.

Examination of sections treated for PR revealed moderate nuclear staining in <10% of tumor cells by chromogen.

Examination of sections treated for P63 revealed positive staining in myoepithelial cell by chromogen.

Examination of sections treated for Ki67 revealed positive nuclear staining in 50 % of tumor cells by chromogen.

DIAGNOSIS: Left breast mass, immunophenotyping :

Duct carcinoma insitu:

- POSITIVE FOR ER SCORE (5/8)
- POSITIVE FOR PR SCORE (4/8)
- Ki67: 50%

PROF. DR. SAMIA GABAL
PROF. DR. MONA SALAH

هبة محمد حسنين



Name: Haiba Mohamed Hussein

Date: 24 March 2026

Pelvi-abdominal ultrasound revealed:

- The liver is enlarged in size measuring about 18.7 cm, with coarse parenchymal echo-pattern. No detectable focal lesions or intra-hepatic biliary radicles dilation.
- Portal vein is patent and of average caliber.
- The gall bladder is averagely distended showing normal wall thickness. No stones or mud seen within.
- CBD is not dilated.
- The pancreas & para-aortic area (visualized parts) are sonographically free.
- The spleen is mildly increased in size (13 cm) with homogenous texture and no focal lesions.
- Both kidneys are within average size with normal site, shape & smooth outlines. They both show normal parenchymal thickness and echogenicity with preserved cortico-medullary differentiation. No sizable renal stones or back-pressure changes bilaterally.
- The urinary bladder is adequately distended with clear urine showing normal wall thickness. No stones, masses or diverticular outpouchings.
- The uterus appears bulky with thickened endometrial stripe measuring about 1.6 cm, showing increased vascularity and no retained contents.
- No gross adnexal masses.
- No ascites or fluid collection.

Opinion

- Enlarged liver with coarse parenchyma, for clinical and laboratory correlation
- Splenomegaly.

Dr. Dina Mohamed, M.Sc



Name: HAYBET MOHAMED HASANIN	Date: 22/01/2026
Age: 38 Years	ID: 518891

Dynamic MRI breast:

Indication:

- Post-operative status of left breast DCIS coming with left breast mass.

Finding:

Marked bilateral background parenchymal enhancement.

I- Left breast:

- An irregular shape solid mass lesion is seen at left breast outer quadrants measuring about 5.7x2.8x6 cm along its axial and CC dimensions respectively, eliciting low T2 and isointense T1 signal with early heterogeneous post-contrast enhancement. Shortest distance to skin is 1cm. No underlying chest wall muscle infiltration.
- ✓ Another similar smaller lesion is seen at central mid-zone measuring 2.7x1.1cm along its axial dimensions with nearby tiny 7mm nodule seen at inner central region.
- ✓ Another early enhancing nodule is seen at retro-areolar region measuring about 6mm in diameter with inseparable anterior linear non-mass enhancement reaching the nipple.
- ✓ Mild mainly peri-areolar and outer region skin thickening.
- Pathologically enlarged left axillary lymph nodes are noted with the largest measuring 3.2x2.3 cm, for sonographic correlation.

II- Right breast:

- No pathological mass or non-mass enhancement.
- Normal skin thickness and contour.
- Normal nipple and areolar complex.
- Intact chest wall.
- Non-specific axillary lymph nodes.

Opinion:

- *Left breast multicentric suspicious mass lesions with ipsilateral pathological axillary lymph nodes (BIRADS 5), for histopathology.*
- *Marked right mammary adenosis (BIRADS 2).*

Dr.Eman Badawy,MD
Dr.Shereen AbdelRaouf,MD

Name: <i>فیت محمد حسنین</i>	Date: 15/12/2025
Age: 38 Years	ID: 28702102504046

*Bilateral mammography with complementary High resolution
Ultrasound Examination*

Indication: left pathologically proven DCIS. Post lactational.

Findings:

- Heterogeneously dense both breasts (ACR c)
- Left UOQ post operative changes in the form scoring.
- Left UOQ peripheral focal asymmetry mounting to mass formation, no calcifications within.
- Right breast showed no speculated masses or clustered microcalcifications detected.

On complementary US:

- Both breast showed post lactational changes in the form of prominent echogenic glandular tissue and mildly dilated ducts.
- Left operative bed shows area of irregular altered parenchyma mounting to mass formation, with internal vascularity on color Doppler. Measuring about 54x32 mm.
- Normal right contour and skin thickness
- Normal right nipple /areola complexes.
- Bilateral nonspecific axillary lymph nodes with preserved fatty hilum.

Opinion :-

- *Right breast normal post lactational changes, BIRADS 1.*
- *Left UOQ operative bed malignant looking area of altered parenchyma mounting to mass formation, BIRADS 6. Dynamic MRI for the assessment of the disease local extension, is recommended.*

Thanks for referral,

Assis. Prof. Menna Hanafy

Dr. Yasmin Essam Khalifa, MD



سليجها سكان الأشعة

Pt. Name: Hibt Mohamed Hasanain	Pt. ID: 330248
Date: 05/11/2025	Pt. Age: 33 Year(s)
Procedure: Mammogram one breast	Ref Doc: ...

Left Breast sonomammography (Craniocaudal & oblique)

Clinical picture: Right painful breast lump.

- Scattered fibroglandular tissue of both breasts (ACR-D).
- Focal dense left outer quadrant asymmetry seen. Complementary Ultrasound revealed: A locoregional focal UQ ill-defined areas of altered echotexture with hyperechogenicity representing infiltrated and inflamed fat lobules, hypoechoic areas in the glandular parenchyma, and associated mild skin thickening, representing focal inflammatory reaction with no localized fluid collection.
- Dilated left retroareolar ducts with clear fluid content (reaching 4 mm in caliber). No intraductal vascular solid component could be detected at current study.
- No spiculated mass is detected.
- No suspicious cluster of micro-calcification is seen.
- Normal skin thickness, subcutaneous fat and nipple areola complex are seen.
- Multiple oval to rounded left axillary LNs, with effaced fatty hila, some shows muffled hila.

Opinion:

- Picture suggestive of UQ focal mastitis & inflammatory reactive axillary LNs, for clinical correlation, close follow up after proper management. (BIRADS 3)
- Mild duct ectasia.

*Much Obliged,,
Dr.Osama safah*